



February 23, 2026

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)
and the Office of the National Coordinator for Health Information Technology (ONC)
U.S. Department of Health and Human Services

Re: RIN 0955-AA13 – Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

To Whom It May Concern:

Netsmart Technologies, Inc. (“Netsmart”) appreciates the opportunity to comment on the Assistant Secretary for Technology Policy (ASTP) Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care. Netsmart is an industry-leading healthcare technology organization empowering behavioral health and substance use treatment providers, home & community-based providers, senior living providers, and specialty practice providers to deliver value-based care to the individuals and communities they serve nationwide.

The Netsmart CareFabric® platform serves as a unified, connected framework of solutions and services for human services, post-acute, specialty practices, payer, and public sector communities. We develop and deliver innovative technology, including electronic health records (EHRs), interoperability solutions, analytics, augmented intelligence (AI), population health management and telehealth solutions and services that assist organizations in transforming the care they deliver. Our experience and client base provide a pragmatic, on-the-ground perspective on the use of Artificial Intelligence (AI) in our communities across care settings, and we appreciate the opportunity to share that insight.

GENERAL COMMENTARY

Netsmart supports a harmonized federal framework that reduces fragmentation by preempting or aligning state rules, protects patients, and enables responsible innovation. We also support payment and programmatic reforms that reimburse high-value AI use cases where evidence demonstrates improved quality, access, and efficiency across clinical and administrative workflows.

A federal framework is necessary to reduce state-by-state fragmentation that imposes significant compliance burdens on providers, and Health IT developers alike. Netsmart supports a federal framework that distinguishes operational tools (e.g., scheduling and

summarization) from clinical tools (e.g., influencing diagnosis or treatment). Standardized transparency requirements that build trust and enable accountability, such as AI model cards, would help improve clinician and patient confidence.

Netsmart supports defined reimbursement pathways for AI-enabled services that measurably reduce burden and improve care quality, including ambient listening, AI-assisted clinical documentation, and automated back-office functions, among other use cases. Payment policies should recognize multi-disciplinary benefits spanning clinicians, administrative staff, billing teams, and care coordinators. Permanent reimbursement pathways could be modeled on telehealth precedents, with safeguards for documentation integrity, audit readiness, and human-in-the-loop oversight. We support aligning reimbursement with value-based care objectives, including the use of AI to reduce avoidable utilization, improve care coordination, and fulfill discrete requirements under programs such as Chronic Care Management, where tasks can be automated yet remain verifiable and clinically supervised.

Netsmart supports targeted federal investment in AI research and development in behavioral health and post-acute care, where fragmentation and privacy constraints have slowed progress. We support the use of de-identified datasets (including registries, vital records, and vetted public health data) for model development and validation. We oppose policies that over-restrict lawful secondary use of appropriately de-identified data in ways that unduly hinder innovation, and we support funding via grants, prospective payment schedules or Center for Medicare and Medicaid Innovation (CMMI) models to reduce the operational costs of AI-enabled tools and to advance health-care-specific models.

SPECIFIC QUESTIONS

WHAT ARE THE BIGGEST BARRIERS TO PRIVATE SECTOR INNOVATION IN AI FOR HEALTH CARE AND ITS ADOPTION AND USE IN CLINICAL CARE?

The primary barriers to innovation in AI for healthcare and the adoption and use of AI in clinical care include cost, regulatory fragmentation, limited trust and clinician acceptance, unclear consent and authorization responsibilities, and constrained access to diverse deidentified data. A state-by-state framework creates inconsistent obligations for data masking, consent, and AI operations, which materially complicates compliance and workflow requirements for health IT developers as well as clinicians operating across multiple jurisdictions. There is also the potential for clinician skepticism in the absence of transparency and human-in-the-loop oversight.

WHAT REGULATORY, PAYMENT POLICY, OR PROGRAMMATIC DESIGN CHANGES SHOULD HHS PRIORITIZE TO INCENTIVIZE THE EFFECTIVE USE OF AI IN CLINICAL CARE AND WHY? WHAT HHS REGULATIONS, POLICIES, OR PROGRAMS COULD BE REVISITED TO AUGMENT YOUR ABILITY TO DEVELOP OR USE AI IN CLINICAL CARE? PLEASE PROVIDE SPECIFIC CHANGES AND APPLICABLE CODE OF FEDERAL REGULATIONS CITATIONS.

For ambient listening and AI-assisted documentation ("ambient scribe") which has already proven its efficacy, we support HHS funding the deployment and implementation of AI tools via expanded reimbursement pathways within prospective payment schedules and other billing methodologies and the creation of grant funding for one-time expenses. The use of technology, such as ambient listening and ambient scribe in clinical care settings, should be reimbursed to incentivize adoption. AI-enabled services support enhanced care delivery and a reduction in administrative burden on providers.

FOR NON-MEDICAL DEVICES, WE UNDERSTAND THAT USE OF AI IN CLINICAL CARE MAY RAISE NOVEL LEGAL AND IMPLEMENTATION ISSUES THAT CHALLENGE EXISTING GOVERNANCE AND ACCOUNTABILITY STRUCTURES (E.G., RELATING TO LIABILITY, INDEMNIFICATION, PRIVACY, AND SECURITY). WHAT NOVEL LEGAL AND IMPLEMENTATION ISSUES EXIST AND WHAT ROLE, IF ANY, SHOULD HHS PLAY TO HELP ADDRESS THEM?

We recommend HHS develop guidance addressing the risks of using general-purpose AI tools outside approved systems, clarifying responsibility and minimum security and privacy expectations. Specifically, HHS should issue guidance that clearly distinguishes consumer-facing mental health chatbots from clinical AI tools used within regulated care settings, recognizing that these technologies serve different functions, have different end users and present materially different risk profiles.

Mental health chatbots are typically designed to provide general wellness support, and engagement directly to consumers and operate outside formal clinical workflows of a regulated provider organization, without clinician supervision or integration into certified health IT systems. By contrast, clinical AI tools are deployed within covered entities, integrated into electronic health records or enterprise platforms, and used by professionals who are trained in their use, to support documentation, care coordination, or clinical decision-making under established privacy, security, and accountability frameworks.

The Clinical AI tools developed by Netsmart are used by clinicians and are not designed to replace them. HHS guidance should clarify that mental health chatbots should not be treated as clinical AI unless they influence diagnosis, treatment, documentation, or care decisions, at which point they should be subject to corresponding clinical governance, transparency, and human-oversight expectations. Establishing functional boundaries based on use and context would reduce provider confusion, protect patients, and support responsible AI adoption across behavioral health and mental health settings.

FOR NON-MEDICAL DEVICES, WHAT ARE THE MOST PROMISING AI EVALUATION METHODS (PRE- AND POST-DEPLOYMENT), METRICS, ROBUSTNESS TESTING, AND OTHER WORKFLOW AND HUMAN-CENTERED EVALUATION METHODS FOR CLINICAL CARE? SHOULD HHS FURTHER SUPPORT THESE PROCESSES? IF SO, WHICH MECHANISMS WOULD BE MOST IMPACTFUL (E.G., CONTRACTS, GRANTS, COOPERATIVE AGREEMENTS, AND/OR PRIZE COMPETITIONS)?

We support establishing federal evaluation baselines for non-device clinical AI grounded in the NIST AI Risk Management Framework. Adoption of a federal framework would also present an opportunity to drive alignment across agencies (e.g., CMS, ASTP/ONC, SAMHSA) on adverse-event reporting, and real-world data collection.

We support funding mechanisms, such as grants, and cooperative agreements targeted to administrative burden reduction, documentation quality, reimbursement efficiency, and use cases in behavioral health and post-acute care. CMMI has an Innovations in Behavioral Health (IBH) Demonstration Notice of Funding Opportunity (NOFO) currently open for solicitation. We recommend CMMI make the AI-enabled behavioral health use cases eligible for immediate reimbursement under the IBH model. This would drive participation in the model and adoption of clinical AI in behavioral health. Furthermore, HHS should consider making behavioral health, post-acute care, and specialty providers of all types eligible for contracts, grants, or cooperative agreements to encourage adoption of clinical AI tools across all sectors of healthcare. We are at a critical juncture of healthcare, and all types of providers should have access to reimbursement mechanisms to enable them to provide the highest quality, most efficient care to all patient populations in the United States.

HOW CAN HHS BEST SUPPORT PRIVATE SECTOR ACTIVITIES (E.G., ACCREDITATION, CERTIFICATION, INDUSTRY-DRIVEN TESTING, AND CREDENTIALING) TO PROMOTE INNOVATIVE AND EFFECTIVE AI USE IN CLINICAL CARE?

Netsmart supports a federal framework adopted consistently across HHS, and its sub-agencies. Additionally, we support the adoption of baseline guardrails related to topics such as safety, transparency, permissible data use, auditability, and human-in-the-loop oversight. We recommend HHS continue to invest in research and development in all sectors of healthcare. We welcome collaboration with HHS and its sub-agencies as you analyze potential use cases for AI in clinical care, especially as it relates to use cases in behavioral health, substance use, post-acute, and long-term care as well specialty practice settings such as physical and occupational therapy.

WHERE HAVE AI TOOLS DEPLOYED IN CLINICAL CARE MET OR EXCEEDED PERFORMANCE AND COST EXPECTATIONS AND WHERE HAVE THEY FALLEN SHORT? WHAT KINDS OF NOVEL AI TOOLS WOULD HAVE THE GREATEST POTENTIAL TO IMPROVE HEALTH, CARE OUTCOMES, GIVE NEW INSIGHTS ON QUALITY, AND HELP REDUCE COSTS?

Ambient listening and documentation have demonstrated robust performance value by improving note quality, reducing administrative burden, and enhancing patient-clinician interaction. 22% of Netsmart behavioral health clients leverage Bells Documentation Assistant, a generative AI solution that helps clinicians create accurate, complete notes faster and reduce documentation time. Users completed 7.2 million notes last year, cutting documentation time by up to 67%. Leveraging technology like this across all Behavioral Health providers in the US could save millions of hours annually. 7% of Netsmart behavioral health clients leverage Bells Virtual Scribe, an ambient AI solution that listens, transcribes and drafts notes in near real time to reduce administrative burden and support patient-focused care. More than 65,000 scribe sessions were completed last year.

We have heard directly from clients how these AI-enabled solutions have reduced administrative burden, streamlined efficiencies, and improved organizational satisfaction. Widespread adoption of these solutions is constrained by funding and reimbursement uncertainty, workforce change management, and variable organizational readiness. We support the development of federal grants and pilots to offset startup costs and to generate publicly available evidence. High-value innovations in AI include tools that reduce administrative burden, strengthen revenue cycle performance, generate accurate referral and handoff summaries, streamline intake, and improve care coordination, with public reporting emphasizing non-proprietary evidence.

Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Within provider organizations, adoption decisions are commonly influenced by executive leadership (e.g., CEOs/COOs), clinical leaders (e.g., CMIOs/CNOs), IT leadership (e.g., CIOs/CTOs), compliance and privacy officers, and finance leaders (e.g., CFOs) given the importance of reimbursement and operational return on investment. The principal hurdles related to the adoption of AI in clinical care include funding streams providers face for the implementation of AI, CMS reimbursement uncertainty, fragmented data stewardship obligations, and conflicting state policies. We support the adoption of a federal framework with accepted standards, and reimbursement pathways to enhance healthcare providers' confidence in the investment and utilization of AI in clinical care settings. To this end, the proposed removal of standard model card data via the HTI-5 proposed rule reduces

certainty, inhibits transparency of data, and inadvertently creates greater confusion for providers trying to understand the breadth and scope of the tools available to them.

WHERE WOULD ENHANCED INTEROPERABILITY WIDEN MARKET OPPORTUNITIES, FUEL RESEARCH, AND ACCELERATE THE DEVELOPMENT OF AI FOR CLINICAL CARE? PLEASE CONSIDER SPECIFIC DATA TYPES, DATA STANDARDS, AND BENCHMARKING TOOLS.

Enhanced interoperability is essential to expanding access to data for AI training, validation, and benchmarking. The ONC certification program aims to advance secure access, exchange, and use of EHI through standardized Application Programming Interfaces (APIs). In addition, the Trusted Exchange Framework and Common Agreement (TEFCA) establishes a nationwide governance and connectivity approach for network-to-network exchange. CMS Aligned Networks are pushing interoperability forward by advancing and expanding the standardized exchange of data. We encourage the administration to continue to focus not only on traditional healthcare settings e.g., hospital, and primary care, but also behavioral health/mental health, post-acute care, long-term care and specialty practice settings and the correlating data. The aforementioned initiatives can reduce fragmentation and help enable secure, scalable pathways for the data access needed to evaluate AI, but only if all patient populations and care settings are included. These community-based providers are often at the bottom of the healthcare funding food chain, and do not have the resources a hospital may have to invest in interoperability, yet they are treating some of the most acute patients in healthcare. Helping to fund their integration with the rest of healthcare providers would greatly improve coordination of care.

We recommend continued focus on advancing data standards to enable AI-generated summaries (a common use case for AI in clinical care) to allow for reliable integration into referral, admission, and financial intake workflows. The United States Core Data for Interoperability (USCDI) establishes a standardized set of interoperable data elements, and HL7 Fast Healthcare Interoperability Resources (FHIR)/US Core provide implementable profiles and APIs that support consistent exchange and ingestion. HHS should also expand access to high-quality public health and population-level data, including immunization registries, which the Centers for Disease Control and Prevention (CDC) describes as confidential population-based systems supporting both point-of-care histories and population surveillance, enabling privacy-preserving research and evaluation across the continuum.

ARE THERE SPECIFIC AREAS OF AI RESEARCH THAT HHS SHOULD PRIORITIZE TO ACCELERATE THE ADOPTION OF AI AS PART OF CLINICAL CARE?

We support HHS investment in behavioral health and post-acute care use cases, de-identification modernization, population health analytics, registry integration, and strategies to reduce the operational costs of large language models and related

infrastructure. We support research on systematic evidence reviews that quantify the costs and benefits associated with AI deployment in clinical workflows.

CONCLUSION

Netsmart appreciates HHS's leadership in advancing responsible AI adoption in clinical care. We welcome continued engagement with ASTP/ONC and other HHS sub-agencies as the agency develops practical guardrails, evaluation approaches, and reimbursement pathways that protect patients while enabling innovation, particularly in behavioral health, post-acute care, long-term care, and specialty practice settings.